

# STATE OF NEW YORK

6758--B

2025-2026 Regular Sessions

## IN SENATE

March 24, 2025

Introduced by Sens. FERNANDEZ, BORRELLO, HARCKHAM, RIVERA, C. RYAN -- read twice and ordered printed, and when printed to be committed to the Committee on Health -- recommitted to the Committee on Health in accordance with Senate Rule 6, sec. 8 -- committee discharged, bill amended, ordered reprinted as amended and recommitted to said committee -- committee discharged, bill amended, ordered reprinted as amended and recommitted to said committee

AN ACT to amend the public health law, in relation to requiring practitioners to discuss certain risks with a patient who is being prescribed a controlled substance or an opioid analgesic

The People of the State of New York, represented in Senate and Assembly, do enact as follows:

1 Section 1. Subdivision 9 of section 3331 of the public health law, as  
2 added by chapter 732 of the laws of 2022, is amended to read as follows:  
3 9. (a) When a patient seeks treatment for any [~~neuromusculoskeletal~~]  
4 condition that causes pain, where a practitioner considers [~~an opioid~~  
5 ~~treatment~~] prescription of any opioid drug which is an initial  
6 prescription for acute or chronic pain and again prior to issuing the  
7 third prescription of the course of treatment, the practitioner shall  
8 consider, discuss with the patient as set forth in paragraph (b) of this  
9 subdivision, and, as appropriate, refer or prescribe non-opioid treat-  
10 ment alternatives, based on the practitioner's clinical judgment and  
11 following generally accepted national professional or treatment guide-  
12 lines, and consistent with patient preference and consent, before start-  
13 ing a patient on opioid treatment. For the purposes of this subdivision,  
14 non-opioid treatment alternatives include, but are not limited to:  
15 acupuncture, chiropractic, massage therapy, physical therapy, occupa-  
16 tional therapy, cognitive behavioral therapy, non-opioid medications,  
17 interventional treatments and non-clinical activities such as exercise.  
18 The practitioner shall inform the patient that some treatments may not  
19 be covered by the patient's health coverage.

EXPLANATION--Matter in italics (underscored) is new; matter in brackets [~~-~~] is old law to be omitted.

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(b) A practitioner shall discuss with the patient, or the patient's parent or guardian if the patient is under eighteen years of age and is not an emancipated minor, the risks associated with the drugs being prescribed, including but not limited to:

(i) the risks of addiction and overdose associated with opioid drugs and the dangers of taking opioid drugs with alcohol, benzodiazepines and other central nervous system depressants;

(ii) the reasons why the prescription is necessary;

(iii) alternative treatments that may be available; and

(iv) the risks associated with the use of the drugs being prescribed, specifically that opioids are highly addictive, even when taken as prescribed, that there is a risk of developing a physical or psychological dependence on the controlled substance, and that the risks of taking more opioids than prescribed, or mixing sedatives, benzodiazepines or alcohol with opioids, can result in fatal respiratory depression.

(c) The department shall develop and make available to practitioners guidelines for the discussion required by this subdivision.

(d) The requirements of this subdivision shall not apply for patients being treated under any of the following circumstances: treatment of cancer; hospice or other end-of-life care; post-surgery treatment immediately following a surgical procedure; or in a medical emergency. For purposes of this subdivision, "medical emergency" means an acute injury or illness that poses an immediate risk to a person's life or health.

§ 2. Paragraph (a) of subdivision 3 of section 3309 of the public health law is amended by adding a new subparagraph (vii) to read as follows:

(vii) "Opioid analgesics" means the medicines buprenorphine, butorphanol, codeine, hydrocodone, hydromorphone, levorphanol, meperidine, methadone, morphine, nalbuphine, oxycodone, oxymorphone, pentazocine, propoxyphene as well as their brand names, isomers and combinations.

§ 3. Subdivision 7 of section 3309 of the public health law, as added by chapter 803 of the laws of 2021, is amended to read as follows:

7. ~~[With]~~ For the first opioid analgesic prescription ~~[to a particular patient]~~ of ~~[an opioid of each]~~ a calendar year that is greater than a one week's supply, for use in a setting other than a general hospital or nursing home under article twenty-eight of this chapter or facility under article thirty-one of the mental hygiene law, or when a practitioner is prescribing a controlled substance to a patient under the care of hospice as defined by section four thousand two of this chapter, the prescriber shall counsel the patient on the risks of overdose, and prescribe an opioid antagonist when any of the following risk factors are present: (a) a history of substance use disorder; (b) high dose or cumulative prescriptions that result in ninety morphine milligram equivalents or higher per day; (c) concurrent use of opioids and benzodiazepine or nonbenzodiazepine sedative hypnotics.

§ 4. This act shall take effect immediately.